

which they supplied the concern with alternative material that the concern must discontinue the production of monomer from scrap Perspex. I.C.I. have explained to us that they consider the use of monomer derived from scrap Perspex in the manufacture of dental products unsatisfactory and likely to damage the reputation of acrylic dental products generally—a view which is disputed by the complainant. This is a technical issue which we are not competent to decide, but we note that towards the end of the war the Ministry of Supply cancelled the instructions for the return of scrap Perspex, and that by 1948 15 per cent. of acrylic denture-base materials were being produced from this scrap.

## CHAPTER 10: THE CASE FOR THE EXISTING REGIME

196. The A.D.M.T. has stated, at some length, its case for maintaining that its practices are not against the public interest and has throughout maintained that the collective enforcement of resale price maintenance is the main aim of the Association. The salient points of its justification for the existence and continuance of an association with this aim are given in this chapter. The Association's explanation and justification of particular aspects of its policy, such as its membership policy, and of particular regulations such as those controlling "excessive services", are given in other parts of the report.

197. The A.D.M.T.'s representatives point out that in the dental goods industry goods are almost invariably sold under a brand name or the name of the maker. In these circumstances, the A.D.M.T. claims, the manufacturer is entitled to maintain the resale prices of his goods. In justification of this claim it puts forward a number of arguments in defence of its practice of resale price maintenance, which it supports by references to the Report of the Greene Committee on Restraint of Trade (1931) and the Report of the Lloyd Jacob Committee on Resale Price Maintenance (1949). From these the main points of the A.D.M.T.'s arguments in favour of resale price maintenance may be summarised as follows:—

(a) In the absence of resale price maintenance, competition among distributors of branded goods tends to result in prices being cut to a level so low that the trade becomes unremunerative and traders cease to stock the goods. This is harmful not only to the distributors but also to the manufacturers and the public.

(b) Admittedly, resale price maintenance raises distributors' margins to some extent. That is its purpose. Manufacturer members, however, would not tolerate unduly high margins.

(c) It is reasonable that manufacturers should be permitted to prevent the price of branded goods being reduced below the level set by reasonable competition.

(d) It may be an incidental disadvantage that low-cost dealers are thereby prevented from passing on to their customers the benefit of any economies they may achieve, but "passing on economies" means price-cutting.

(e) There is no evidence that resale price maintenance impedes the introduction of improved methods of distributing dental goods.

(f) If resale price maintenance is a good thing, it is unreasonable to say that its application should be left to individual manufacturers. Individual systems are difficult and expensive to operate; collective systems are simpler, cheaper and more effective.

(g) Price maintenance was adopted in many trades as a result of grave and real apprehension in the minds of distributors over a period of fifty years or more. This suggests the conclusion that something in economic conditions compelled people to come together.

(h) Resale price maintenance can be a defence against high prices in times of shortage as it is against price-cutting in times of slump.

(i) The maintenance of freedom of contract in commercial affairs is a matter of public interest, and no case has been made out for exceptional treatment of resale price maintenance.

198. The argument for the collective enforcement of resale price maintenance in the dental goods industry is based on the assumption that resale price maintenance is desirable and that "collective methods can achieve it while the efforts of individual manufacturers each striving unsuccessfully to protect their own prices cannot". The reasons given by the Association for the statement that individual manufacturers cannot without collective measures enforce their end prices are:—

(a) that there would have to be an individual contract between manufacturers and each of their distributors and that, owing to the nature of the dental trade, this contract would have to be very complicated and to include many of the existing rules of the Association;

(b) that it would be difficult to detect breaches of the contracts because goods are not sold over the counter to the public and it is doubtful whether the distributors would allow manufacturers (as distinct from the Secretary of the Association) to inspect their books;

(c) that manufacturers could not afford to enforce their contracts with distributors by refusing to supply, if the only result would be that the distributor would transfer his custom to another manufacturer; and

(d) that accordingly the manufacturer's only remedy under an individual contract is to obtain an injunction or interdict, which is a costly and slow procedure and which must also be wasteful, since in the case of a deliberate price-cutter several manufacturers would have each to take legal action against a single dealer.

199. The A.D.M.T. has explained to us that resale price maintenance can be collectively enforced by either the "black list" or the "white list" method, i.e., by manufacturers agreeing either to withhold supplies from certain named dealers or to supply certain named dealers only. The A.D.M.T. decided to adopt the second method, based on the agreement by manufacturer members to supply only through dealer members. The Association claims that this method has the advantages that the detection of breaches is made possible by inspection of books by impartial persons (such as the A.D.M.T.'s Secretary or accountants), that the penalty can be made to fit the circumstances of each case, so that many comparatively trivial breaches are effectively dealt with by a reprimand or a small fine, and that the number and kind of distributors can be controlled. The A.D.M.T. adds that the pressure of opinion within the Association is strong enough to ensure a certain standard of conduct on the part of its members, the standard exacted being accepted in exchange for the protection afforded by membership.

200. The other half of the exclusive dealing arrangement, i.e., the undertaking by dealer members to buy essential dental goods from manufacturer members only, was originally explained (see para. 43) as a simple *quid pro quo* for the manufacturers. The same explanation was offered by a representative of the Association in evidence before the Committee on

**Resale Price Maintenance.** Now, however, the A.D.M.T. has explained to us this part of the arrangement in the following terms:—

“If a dealer undertakes by virtue of his membership of the Association to abide by the Association's Rules, he ought to be protected against unfair competition from persons who are not members of the Association whether they are manufacturers or not, and the rule requiring dealers not to buy goods from non-members is a necessary part of the price maintenance structure for the protection of the general body of dealers against such competition by a manufacturer who is not a member.”

201. An Association based on exclusive dealing must possess the power to expel members and refuse applications for membership. The A.D.M.T. takes the view that it is not in the public interest that all applicants who seek admission should be automatically elected and it claims that the discretionary power of the Council has, in fact, always been exercised reasonably and with a full sense of responsibility, although it admits mistakes have been made.

202. According to the A.D.M.T.'s representatives, the A.D.M.T. was formed to combat the effect of fifty years of price competition between dealers which had gradually brought the industry to the brink of disaster. They claim that by 1923, price-cutting had become so flagrant that very few concerns would have been able to survive, had the A.D.M.T.'s system of collective resale price maintenance not been introduced. Should resale price maintenance become ineffective in the dental goods industry (and the A.D.M.T. for the reasons given is convinced that it would, if the powers of boycott should be disallowed), the A.D.M.T. maintains that there would be a rapid return to unrestricted price-cutting and the chaotic conditions which ruled when the Association was formed. That would mean, in its view, the survival of two classes only of dealers; the financially strong who would be able to go on cutting prices until their competitors were eliminated, and very small traders giving no service and having practically no overhead expenses. These small traders would deal only in pre-packed goods and therefore the distribution of all other dental goods would become concentrated in fewer and fewer hands. The A.D.M.T. thinks that in the end the manufacturers would either have to accept dictation from a small monopoly group of the first class of dealers or each manufacturer would have to set up his own distribution section. The waste involved in the latter course is, in the A.D.M.T.'s view, obvious.

203. In any event, whichever course of action prevailed, the A.D.M.T. asserts that there would be a general disorganisation of the whole manufacturing and distributing structure of the trade with a consequent failure to provide the quality of product and service which is desired by and at present afforded to the profession. The A.D.M.T. has laid emphasis, as have the dentists themselves, on the special and varied needs of the profession, which in turn necessitate a peculiarly specialised industry and distribution system. Dental goods, it is convinced, can be satisfactorily distributed only by technically qualified and experienced persons. The present distribution system which has evolved out of the long experience of those engaged in this trade is, in the opinion of the A.D.M.T. members, the most satisfactory system for this particular trade.

204. The A.D.M.T. stresses the fact that its arrangements do not include any system for the fixing of common prices by manufacturer members and that, although dealers are bound to observe the retail prices laid down by each manufacturer member for his goods, competition exists between the manufacturers, each of whom is free to fix his own prices and margins. As has been pointed out in para. 64, the maximum cash and quantity discounts laid down in the Regulations limit the manufacturer's discretion in settling these discounts for his products. The Association says that the control of

quantity rates and cash discounts has the object of ensuring that the concessions allowed should fairly represent the advantage gained from prompt payment and quantity sales, and that quantity discounts should not be used as a device to attract custom and encourage over-purchasing.

205. While the A.D.M.T. is confident that its past history and current activities amply justify its continued existence in its present form, it has offered to make certain changes in the Rules and Regulations. The A.D. Co. has also put forward certain suggestions as to the form which any dental trade association should take. Both sets of suggestions are summarised in Appendix 28.

206. The proposals made by the A.D.M.T. do not all tend towards the liberalisation of the Association's rules—they include, for example, the suggestion that in future members should be prohibited from selling to non-members who are known price-cutters, even at retail prices, except on a direct undertaking by the purchaser not to sell above or below the maintained price—but they include the offer to give up certain penalties and to institute some kind of appeal procedure for applicants for membership based on the publication of certain of the requirements for membership which are now purely matters for the discretion of the Council.

207. The A.D. Co. suggests that the rules and regulations of a dental trade association might well be strictly limited to what is necessary to maintain manufacturers' end prices; that the association should not have any rules which appear to interfere in any way in the fixing of end prices and trade discounts, that membership should be open automatically to all *bona fide* dealers; that the only penalties should be expulsion or suspension from the association and small fines; that the rules should not hinder or appear to hinder the acquisition as between dentists of secondhand equipment; and that, if a system of supplies in quantity for hospitals is capable of development without a substantial increase in administrative costs, no rule should be permitted which would prevent hospitals obtaining goods in quantity direct from manufacturers.

208. Some of these suggestions are in line with our own views, but they would leave the essential features of the system unchanged.

## **CHAPTER 11: CONCLUSIONS ON THE PUBLIC INTEREST IN RELATION TO THE CONDITIONS AND PRACTICES FOUND TO PREVAIL IN THE INDUSTRY**

209. We now come to item 3 of our terms of reference, namely, our duty to report "whether the conditions in question or all or any of the things done as foresaid operate or may be expected to operate against the public interest". Under the terms of the Act we must have regard, amongst other things, to the matters set out in Section 14 of the Act. Before proceeding to this duty, we would emphasise that, whereas the conditions found to prevail are matters of fact, our conclusions on public interest must necessarily be matters of opinion. We are not asked to say whether the conditions have in the past operated against the public interest, but whether they do so now or are likely to do so in the future. We have felt that one of the most important guides in this task must be the use and effect of the industry's practices and policies in the past and we have, therefore, described these at length. We have to bear in mind, as regards the future, that the National Health Service has created a new situation and, as regards the past, that trading conditions have not been normal in the last few years.